

Our Ref: (T) mb/ss/ 4037830

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

CG89, G level, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses :

Crohn's disease

Ileocolonic resection

Medications :

Humira 40 milligrams every other week (January 2017)

Azathioprine 50 milligrams daily (currently on hold)

Previous Medication: Infliximab loss of response.

I reviewed Alice in clinic today together with her parents. It has been some time since we first reviewed Alice. Back in January she underwent colonoscopy which shows disease recurrence at the anastomosis. At that time we have made the decision to discontinue Infliximab infusion and switch to Humira. Unfortunately she had failed to attend a couple of appointments and had not had any blood monitoring I therefore had to put her Humira account on holds. It has been a couple of weeks since she has last had her Humira and has noticed a significant effect with not having it. Abdominal pain has slightly increased in frequency as her main concern over the past couple of days. She has responded extremely well to the medication and it is clear that we need to continue and I have stressed the importance of having a periodic review and regular blood monitoring.

Her fatigue is an ongoing issue and she is struggling with this. It may be related to her azathioprine and in view of repeated viral infection I have suggested that she hold off taking the azathioprine for a month or so to see if this improved her symptoms in anyway. If it does not then I have stressed to Alice that she will need to restart taking it. Meanwhile she will continue on Humira every other week. I suspect this is going to need to be for the long term. Alice is also struggling with slight nausea and she was asking if anybody else was able to administer her injection. I think she did approach her boyfriend on one occasion how this led to arguments and I have suggested that her boyfriend is an ideal person to be administering her injection. I have asked her if she can approach the practice and your surgery to see if they are able to administer her injections once a fortnight for a month. I will see her for injections just to see if this does decrease her anxiety. I am more than happy to fax over a Humira prescription if required. Failing that I will ask the healthcare home team to arrange a home visit for one or two of the injections. Past experience shows that taking the pressure of administering the injections over the next few weeks usually reduces their anxiety of the needle phobia to some extent. I have very much hope you are able to support Alice with this.

Meanwhile she informs me that there is a possibility of her move to Spain at which point we will obviously have to work out the logistics of her having regular blood monitoring and indeed getting access to the medication. She is going to keep me updated with her plan. Meanwhile we will see her for a review in clinic for a year's time and she has my secretary's contact number should she run into any problems.

With kind regards

Yours sincerely

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Sister Marion Bettey RGN BSc (Hons)

Biologics CNS Independent Nurse Prescriber